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PRACTICE POINTER

Identification and management of child abuse and neglect

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What you need to know

- Child maltreatment may present with subtle features, and a history of abuse or neglect is not typically provided
- Perform a full skin and intraoral examination in pre-mobile infants, consider occult injury screening, and use age- and situation-appropriate questions to help identify maltreatment
- If child sexual abuse is suspected, limit the number of physical assessments to minimise trauma to the child and contact the local sexual assault (referral) centre in a timely manner
- Given the potential for legal proceedings, ensure documentation is clear and objective, using quotations, drawings, and photographs when indicated

Child abuse and neglect, collectively known as maltreatment, is common, and most healthcare providers who care for children will encounter some form of maltreatment in their practice.¹ Common presentations include physical abuse, sexual abuse and commercial sexual exploitation of children, and neglect.

In this article, we make recommendations for frontline healthcare providers on identifying child maltreatment and making evidence based assessments in primary care and emergency department settings. While presentations of maltreatment may be similar internationally, the subsequent investigation and management recommendations vary according to culture, legislation, income classification, and resources. We discuss broad principles with respect to recognition and subsequent escalation and reporting to child welfare authorities or social services, largely in keeping with North American and European settings. If raising concerns about a child's welfare, we recommend all healthcare providers consult local laws and statutes in the first instance.

What is child maltreatment?

Child maltreatment is the physical, sexual, and psychological/emotional abuse, and neglect of infants, children, and young people aged 0-17 years by parents, caregivers, and other authority figures, as defined by the World Health Organization (box 1).⁶ Emotional abuse and fabricated or induced illness in children are forms of child maltreatment, but beyond the scope of this article. Child maltreatment may present acutely or may be longstanding, and multiple forms can co-occur.

Box 1: Types of child maltreatment

- *Physical abuse*—The intentional use of physical force against a child that results in, or has a high likelihood of resulting in, harm for the child's health, survival, development, or dignity¹
- *Sexual abuse*—The involvement of a child or a young person in sexual activity that he or she does not fully comprehend and/or is unable to give informed consent to, or that violates the laws of society²
- *Emotional abuse*—A pattern of behaviour that attacks a child's emotional development and sense of self worth, including excessive, aggressive, or unreasonable demands that place expectations on a child beyond his or her capacity. Emotional abuse includes repeatedly criticising, belittling, insulting, rejecting, ignoring, or isolating a child, and may also include exposure to domestic violence³
- *Neglect*—An act of omission in the care, or failure to provide necessary care, leading to potential or actual harm⁴
- *Fabricated or induced illness in children*—A form of child maltreatment caused by a caregiver who falsifies and/or induces a child's illness, leading to unnecessary and potentially harmful medical investigations and/or treatment. This condition can result in significant morbidity and mortality⁵

Child maltreatment is common and occurs in all populations. Reported rates vary, depending on the population studied and data source (for example, data from child protective services include only cases that have been reported and investigated, whereas population data stem from asking adults how they were treated as children). In a review of meta-analyses of 244 publications and 551 prevalence rates globally, the overall estimated prevalence rates for retrospective self-report studies were 127/1000 for sexual abuse (76/1000 among boys and 180/1000 among girls), 226/1000 for physical abuse, 363/1000 for emotional abuse, 163/1000 for physical neglect, and 184/1000 for emotional neglect.⁷ For research on sexual abuse, studies in high income countries, particularly North America, and research using self-report measures represent most of the literature used in this review. Data from low and middle income countries remain limited.

Various risk factors for maltreatment have been identified (box 2). Child maltreatment may occur in the absence of known risk factors, and clinicians should consider their own biases, which may affect decision making.¹⁰ Social risk factors, such as socioeconomic status, and behaviours of caregivers in the healthcare setting are insensitive, subjective, and potentially biased when used to consider whether concern should be raised for maltreatment.¹¹ Studies

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have highlighted bias and racial overrepresentation in the medical assessment of child abuse and neglect and in reporting to child welfare authorities or social services.¹²⁻¹⁴ Consideration of these risk factors may instead be helpful in the holistic assessment and discharge planning for the child.

Box 2: Selected risk factors for child maltreatment^{8 9}

- *Child factors*—Medical complexity, intellectual disability, behavioural concerns or neurological disorder, identifying as LGBT+
- *Caregiver factors*—Personal history of maltreatment, criminal activity, substance misuse, difficulty bonding with the child, high stress levels
- *Family and community factors*—Family violence, isolation, lack of support, poverty, socioeconomic instability

How do children present?

Children do not usually present to a healthcare provider with maltreatment as the primary concern. Presentations vary across settings, ranging from asymptomatic children disclosing abuse or neglect, to clinicians recognising unusual bruising, to infants presenting acutely unwell or with life-threatening injuries.

Physical abuse

Physical abuse in infants may present as a sentinel injury. Sentinel injuries are minor, visible injuries in pre-mobile infants (typically less than 6 months old), and include bruising and intra-oral injuries such as torn frenula. They are often found during thorough examination for an unrelated reason. While these may seem insignificant in isolation, they raise concern for possible concurrent occult injury, such as fractures or intracranial haemorrhage, or future serious injury.¹⁵ The importance of sentinel injuries as an indicator of future injuries is highlighted in a retrospective, case-control study of 200 infants with serious injuries (such as head injury, abdominal trauma, fractures, or burns) and a high concern for abuse.¹⁶ About 28% of these infants had a previous sentinel injury, compared with none in the control group of infants determined not to have been physically abused. Healthcare providers were aware of the sentinel injury in 42% of cases, suggesting an opportunity for earlier medical intervention to prevent future serious outcomes.

In all children presenting for medical care with injuries, consider the features in [box 3](#). These features are not diagnostic for physical abuse but suggest that further consideration for child maltreatment, or an underlying medical condition, may be required.

Box 3: Injury features warranting further assessment^{17 18}

- Incompatibility with development stage, for example, injury in a pre-mobile infant who is unable to self injure
- Inadequate explanation for injury regarding mechanism, severity, symptoms, or timing

- Patterned skin injuries that raise concern for injury with an object of that shape or outline
- Bruising in unusual locations, such as the ears, neck, abdomen, or buttocks
- Multiple or extensive unexplained injuries
- Rib and classic metaphyseal fractures in infants, which are highly associated with inflicted injury¹⁹
- Supervision concern or delay in seeking care, although this may be subjective

Examination

At minimum, complete a skin and intra-oral examination in pre-mobile infants to decrease the chance of missing injuries.^{15 16} Head circumference measurements in infants with non-specific neurological symptoms (such as vomiting without diarrhoea, irritability, abnormal movements, apnoea) can help detect possible intracranial causes, including haemorrhage from maltreatment.²⁰ As in routine paediatric care, an increase in head circumference percentiles that is clinically concerning, or an isolated measurement outside of the normal range, should prompt further assessment.

Bruising and injuries may appear differently in children and young people with darker skin tones and can be difficult to detect.²¹

Screening

The usefulness of universal screening tools to identify physical abuse remains debated. The risk of falsely identifying cases and causing distress from involving child welfare authorities or social services must be balanced against the chance of missing cases. A 2019 systematic review concluded that—because of concern of bias, gaps in literature, and rates of false positives—existing universal screening tools to identify maltreatment are not currently recommended.¹⁷ However, this was based on low quality evidence.

Since 2019, the literature on the development and validation of clinical decision rules has grown. One such tool is the TEN-4-FACESp bruising clinical decision rule, where TEN-4-FACESp is an acronym for regions and patterns of bruising in children less than 4 years old presenting to the emergency department ([fig 1](#)).²² If bruising in any of the three components is present without a reasonable explanation, consider evaluation of child abuse and/or specialist advice. In one cross-sectional study of 2161 children, having three or more out of 10 components was reported to have a 95.6% (95% CI 93.0% to 97.3%) sensitivity and 87.1% (85.4% to 88.6%) specificity for distinguishing abuse from non-abusive trauma based on the characteristics of the bruising, though determination of abuse was by an expert panel convened for the purpose of the study.²² This rule is to be used as a screening tool, and should not be used to diagnose abuse.

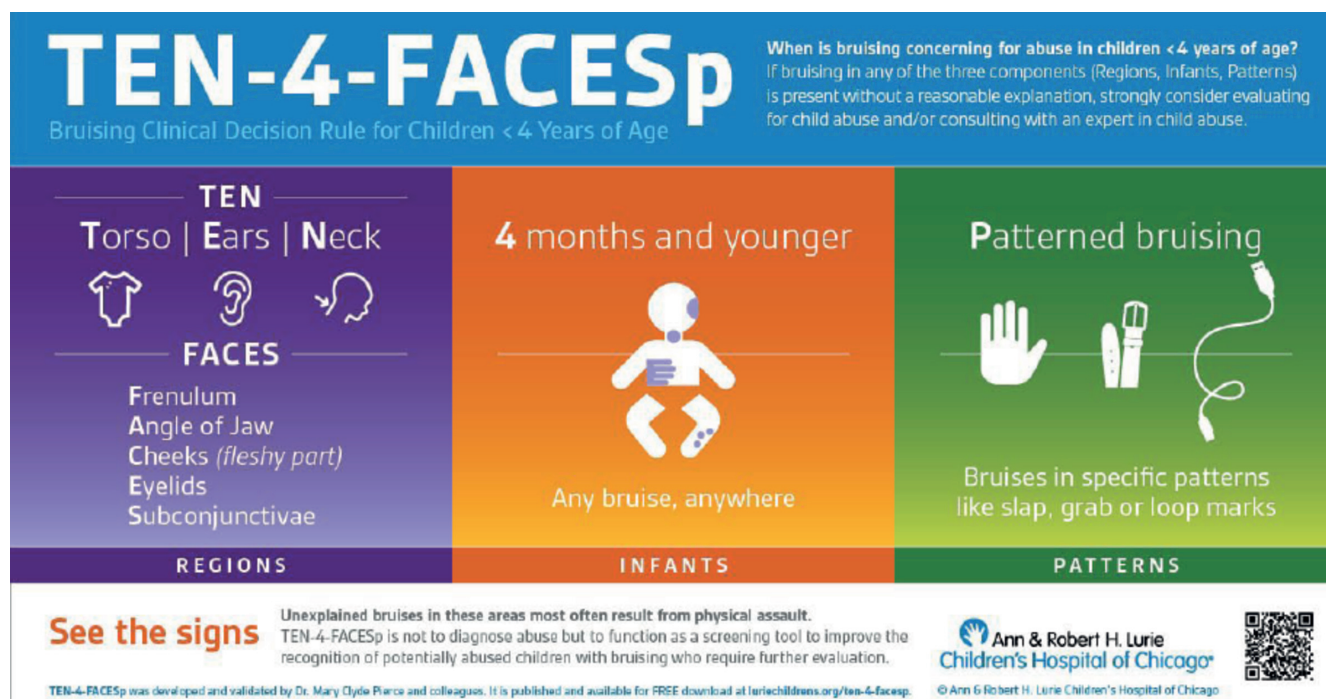


Fig 1 | The TEN-4-FACESp rule. If bruising in any of the three components (regions, infants < 4 months, patterns) is present without a reasonable explanation, consider evaluation of child abuse and/or specialist advice. Reproduced from the Stanley Manne Children's Research Institute <https://research.luriechildrens.org/en/community-population-health-and-outcomes/smith-child-health-outcomes-research-and-evaluation-center/tricam/ten-4-facesp/>

Clinical decision algorithms for identifying abusive head injury have also been developed. These are different from clinical decision tools for mild head injury as they do not rely on a history of traumatic injury. These tools may help support the clinician in considering the need for neuroimaging in children with non-specific neurologic symptoms who appear well.²⁰ Further research is required to understand the role of such tools in routine practice.

Sexual abuse

Children who have been sexually abused or assaulted often do not have genital findings or injuries at the time of assessment. This may be due to the nature of the abuse, the ability of genital tissues to stretch without injury or heal quickly without residual findings, and delayed disclosures from children.^{23,24} The absence of symptoms or injuries does not exclude the possibility of sexual abuse.²³ In box 4, we outline how concerns for suspected child sexual abuse may be raised with frontline care providers.

Box 4: Child sexual abuse concerns in the medical setting

- A child makes a disclosure to their caregiver, who brings them to a healthcare provider. This disclosure may be of recent sexual abuse or historic sexual abuse that the child discloses years later. This child should be referred to an appropriate specialist for medical assessment and treatment
- A child has symptoms in the absence of a disclosure that raises caregiver concern for sexual abuse but may be explained by medical conditions. A caregiver's personal history of trauma may affect their response. Isolated genital symptoms are commonly due to medical conditions.²⁵ These include:
 - Vaginal discharge due to vulvovaginitis, streptococcal infection, foreign body (such as toilet paper)
 - Vulval redness due to vulvovaginitis, diaper dermatitis, or lichen sclerosis

- "Vaginal" bleeding due to urethral prolapse, labial adhesions, lichen sclerosis, or endocrinological conditions
- Variations in the size of the hymenal opening. There is no standard size for a hymenal opening, therefore hymens with "larger" openings are normal
- Perianal or anal redness, irritation, or fissures due to constipation
- Dilation of the anus due to constipation
- Sexualised behaviours, which occur on a spectrum and may be normal at a certain age²⁶
- Uncommonly, a child presents with symptoms or injuries that raise more specific concern for possible sexual abuse or assault.²³ These include:
 - Unexplained acute or healing anogenital injury
 - Unexplained sexually transmitted infections
 - Pregnancy

When child sexual abuse is suspected, limit the number of physical assessments to minimise trauma to the child from repeated genital examinations.²³ Contact sexual assault centres in a timely manner for advice. When appropriate, defer to specialist examination by a trained professional to ensure that the most experienced provider completes the examination (and collects evidence of sexual assault when indicated).²³

Commercial sexual exploitation of children

Commercial sexual exploitation of children is a subset of sexual abuse that includes sex trafficking and pornography.²⁷ It also includes cyber enticement, a form of exploitation due to the advent of increasing social media use that involves communicating with someone believed to be a child via the internet with the intent to

commit a sexual offense or abduction.²⁸ Risk factors include high risk sexual behaviours, substance use, running away from home, police involvement, violence from and to others, being precariously housed or housed in a shelter, group or foster home, and other forms of maltreatment.¹ Young people may not identify themselves as victims, but studies have shown they present for healthcare needs.²⁷

Assessment

In young people who have been identified as high risk through their social history, we recommend assessing for risk of trafficking using Greenbaum's six-item questionnaire (box 5). In the original cross-sectional study, young people aged 12-18 years who identified as victims of commercial sexual exploitation (n=25) were compared with similar aged patients with allegations of acute sexual assault or abuse without evidence of commercial sexual exploitation (n=83). The questionnaire was constructed based on significant differences between the two groups, and two positive answers to the questions posed were found to have a sensitivity of 92% and specificity of 73% (ranges not reported) for commercial sexual exploitation.²⁷

Box 5: Assessing for commercial sexual exploitation of children

Greenbaum's six-item questionnaire assessing for commercial sexual exploitation in high risk young people²⁷:

- Have you ever used alcohol and/or drugs?
- Have you ever run away from home?
- Have you ever been involved with law enforcement?
- Have you ever broken a bone, had traumatic loss of consciousness, or sustained a significant wound?
- Have you ever had a sexually transmitted infection?
- Have you had sexual activity with more than five partners?

After two positive answers, based on our clinical experience, we recommend asking²⁹:

- Has anyone ever asked or pressured you to engage in any type of sexual act?
- Do you know of anyone who had to, or have you ever had to, exchange sex for money, food, shelter, to survive, or to get something you needed?
- [If the person discloses that they have run away from home] How do you get by when you run away from home?

When commercial sexual exploitation of children is suspected, referral should be made for a coordinated, multidisciplinary assessment and planning that ensures the safety of the child, involves relevant authorities, and includes ongoing medical assessments and mental health support.

Neglect

Types of neglect include physical, emotional, supervisory, medical, dental, and educational neglect. The determination of neglect can be challenging, as adequacy of care for a child may exist on a spectrum and is influenced by societal norms. Consider approaching neglect as a symptom to which the underlying cause should be sought (table 1).⁴ For example, if a child presents with poor hygiene and ill-fitting clothes, consider whether this may be due to poverty. Subsequent management may include engaging social workers, community resources, or ongoing education for the family as opposed to immediately viewing the situation as a result of maltreatment. However, when there is a repeated pattern of behaviours that is resistant to education or social interventions, consider maltreatment.

Table 1 | Concerns for possible neglect in the medical setting, and contributors and suggestions to explore^{4 18}

Concerns	Possible contributors	Suggestions
- Poor hygiene - Ill-fitting clothes	- Poverty - Multi-child family	- Engage social worker - Community supports
- Recurrent or persistent infections such as scabies - Head lice	- Poverty - Infestation in entire household, or community living (eg, shelter) - Health literacy concerns regarding treatment	- Treating close contacts - Education
Growth faltering	- Contributing medical conditions - Poverty - Inability to afford formula	- Medical work-up for causes of faltering growth - Assessment for macro or micronutrient deficiencies - Dietitian and/or lactation consultant referral - Providing samples of formula
- Multiple missed health appointments - Delayed presentation for medical concerns	- Transportation difficulties - Caregiver does not have a phone - Caregiver unable to take leave from job - Competing family priorities	- Providing tokens for public transportation - Emailing/mailling appointment reminders - Virtual appointments if appropriate - Providing sick leave notes
Medical non-adherence	- Alternate health beliefs - Prior trauma or distrust in the medical system - Health literacy concerns - Language barrier - Inability to afford treatment - Behavioural difficulties in child preventing treatment	- Education - Deciding on an alternate safe medical option - Engaging other family members or religious health leaders for support - Caregiver capacity assessment - Use of interpreter - Providing samples of medications^{37 38}
Supervision concerns (general or resulting in injury)	- Multi-child family with single caregiver - Competing family mental or medical health concerns	- Adequacy of supervision exists on a spectrum - Consider harm or risk of harm to child to determine next steps

Framing the concerns of neglect around whether there is urgent risk of harm, risk of significant harm, and all efforts for education and optimising care have been exhausted is helpful when considering reporting to child welfare authorities or social services.⁴

Healthcare providers can assist by completing thorough paediatric assessments for development, dentition, vision, growth patterns, diet, routine vaccinations, and school attendance and providing close medical follow-up.

How should I ask about child abuse or neglect?

At initial assessment, healthcare providers should obtain sufficient information to determine next steps, including immediate medical management, possible referral to a paediatric or safeguarding specialist, and consideration of reporting to child welfare authorities or social services when indicated. Depending on local resources and regulations, it is best practice for trained specialists to formally interview children. In some areas, interviewing may be done only by child welfare, social services, or police. Elsewhere, healthcare providers may undergo training to perform forensic or investigative interviews.

Asking a child

Gathering information from a child varies depending on their age and developmental stage. Ask open ended questions using appropriate language. Do not ask leading questions, because in younger children these may prompt answers that do not reflect actual events. Depending on the age of the child, there may also be limited understanding of simple or vague terms in the context of maltreatment (for example, if a clinician discusses vaginal or anal penetration using the word “inside,” which a child interprets as meaning contact between the labia or buttocks). Consider who else is in the room and their influence on what the child may or may not say. Recall that it is important to stay within your medical scope and leave detailed interviewing to trained professionals. In cases where a child has already made a disclosure of sexual abuse, direct questioning of the child is not needed by a healthcare provider.²³

Asking a young person

Social histories focused on young people such as HEADSSS (home, education, activities, drugs, sexuality, suicide, safety) may identify concerns.³⁰ To encourage information sharing, be non-judgmental, meet with the young person alone, and explain confidentiality and its limits. Limits to confidentiality vary by jurisdiction but include suicidal or homicidal intent typically, and abuse or neglect concerns that meet the clinician’s duty to report.

We recommend using a trauma-informed approach, which recognises the widespread impact of trauma in an individual’s life, promotes a safe environment focused on a patient’s strengths, and empowers them to have control in decision making.^{31 32} For young people, this can include being sensitive to their distress, following their lead as to what they want to address at each encounter, and recognising that it may take several encounters to develop rapport.³¹ Consider whether necessities, including food, hygiene, and shelter, need to be addressed before any further medical or mental health therapies.³³

Asking a caregiver

Caregivers may have concerns to share regarding maltreatment of their children. Ideally, use a private space for these discussions and consider the impact of the conversation on any other children in the room.

Caregivers may have a range of reactions when asked about the possibility of someone harming their child. Focus on obtaining the necessary information to inform your next steps in an objective and non-accusatory manner (box 6). This may include injury mechanism, developmental ability and symptoms of the child, and risk factors for related medical conditions.

Box 6: Suggested phrases when speaking with a caregiver about the possibility of maltreatment

- “I noticed this [injury]”

- “Can you tell me about this [injury]?”
- “As a physician, I am legally mandated to make a report to child welfare authorities (social services) when...”
- “It is standard hospital protocol that I refer to a specialist or child maltreatment paediatrician when...”
- “I am concerned about the unexplained/unusual/serious injury in your infant, so I am referring you to a specialist/informing the child welfare agency so they can assist us in understanding what happened”
- “Given your concerns about [any form of child maltreatment], I am making a report to the child welfare agency (social services). They are the best people to explore this and assist in finding out what happened”

How should I respond to concerns?

We recommend healthcare providers abide by local policy and processes. In our area of practice, this involves a primary care practitioner or generalist working in the emergency department consulting a designated child maltreatment or safeguarding practitioner or, if appropriate, sexual assault centre in the first instance. Response may eventually involve authoring a report to local child welfare agencies or social services.

An initial medical work-up for possible physical abuse can occur concurrently with escalating concerns to child welfare authorities or social services.^{10 11} Examples include assessing for medical conditions that may be mistaken for or contribute to injuries (such as bleeding disorders, bone health conditions, metabolic or genetic conditions) and concurrent occult injury screening (such as skeletal survey in children <2 years old, head imaging in children <1 year old, ophthalmology assessment in the presence of intracranial injuries, intra-abdominal screening with liver and pancreatic enzymes). When possible, assessment is best guided by specialists and should adhere to published guidelines.^{19 25 34 35}

Given the legal complexities in cases of suspected child sexual abuse, it is particularly important for examination and management to be guided by specialists. If unavailable, guidelines for medical management are well established and include recommendations on testing and prophylaxis for sexually transmitted infections, pregnancy testing, emergency contraception, and vaccination for hepatitis B and human papillomavirus (HPV).^{2 23 24}

For all types of abuse, where appropriate, consider referral to mental health and other community based supports for children and their caregivers.

How should I document in the medical record?

Document your assessment and discussions in a neutral, clear, and objective manner, as clinical documentation may be used in legal proceedings (box 7, table 2). Describe skin injuries in detail, including colour, size, and location. A body map can complement the descriptions.

Box 7: Suggestions for objective documentation^{17 36}

- Document who provided the information to you. If multiple people provide information, document who said what
- Describe how that information was obtained; that is, did you ask directly, or did the person disclose it unprompted?
- Use quotation marks to document verbatim disclosures provided to you
- Document caregiver subjective concerns and your objective findings

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- Avoid interpreting the appropriateness of the emotional state of caregivers. Different people may react to situations in different ways, particularly if it is shocking or traumatic
- Document any consents you obtained clearly, whether verbal or written. Consent should be sought for your assessment in general,

- but is particularly important for genital examinations, medical photography, and sharing of medical information. Ensure that it is the custodial guardian providing consent
- Document the name, agency, and contact information of the child welfare authorities or social services if available

Table 2 | Samples of written documentation

Inadequate documentation	Improved documentation	Comments
6-week-old male with shaken baby syndrome and bruise on right leg	6-week-old male with skull fracture, bilateral subdural haemorrhages, and 2×3 cm green bruise on anterior right thigh, currently unexplained	This avoids outdated and inaccurate terminology such as shaken baby syndrome and includes additional descriptors of the bruise
The caregiver was inappropriately calm and did not seem upset when hearing about the injuries	The caregiver had a flat affect	This avoids subjective interpretation of how the caregiver appeared. Note that a flat affect can have multiple causes, including shock
16-year-old rape victim	16-year-old disclosing sexual assault	This uses objective terms and avoids labelling as victim

Medical photography can be helpful but requires consent and secure storage. Concerns should be documented objectively without overinterpretation. Outdated terminology such as “shaken baby syndrome” should be avoided as it presumes mechanisms or intents. Document communication with child welfare authorities or social services.

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How patients were involved in the creation of this article

An individual with lived experience of child maltreatment contributed to reviewing and editing this article. They have been involved personally and professionally with the Lotus Health Clinic at the Hospital for Sick

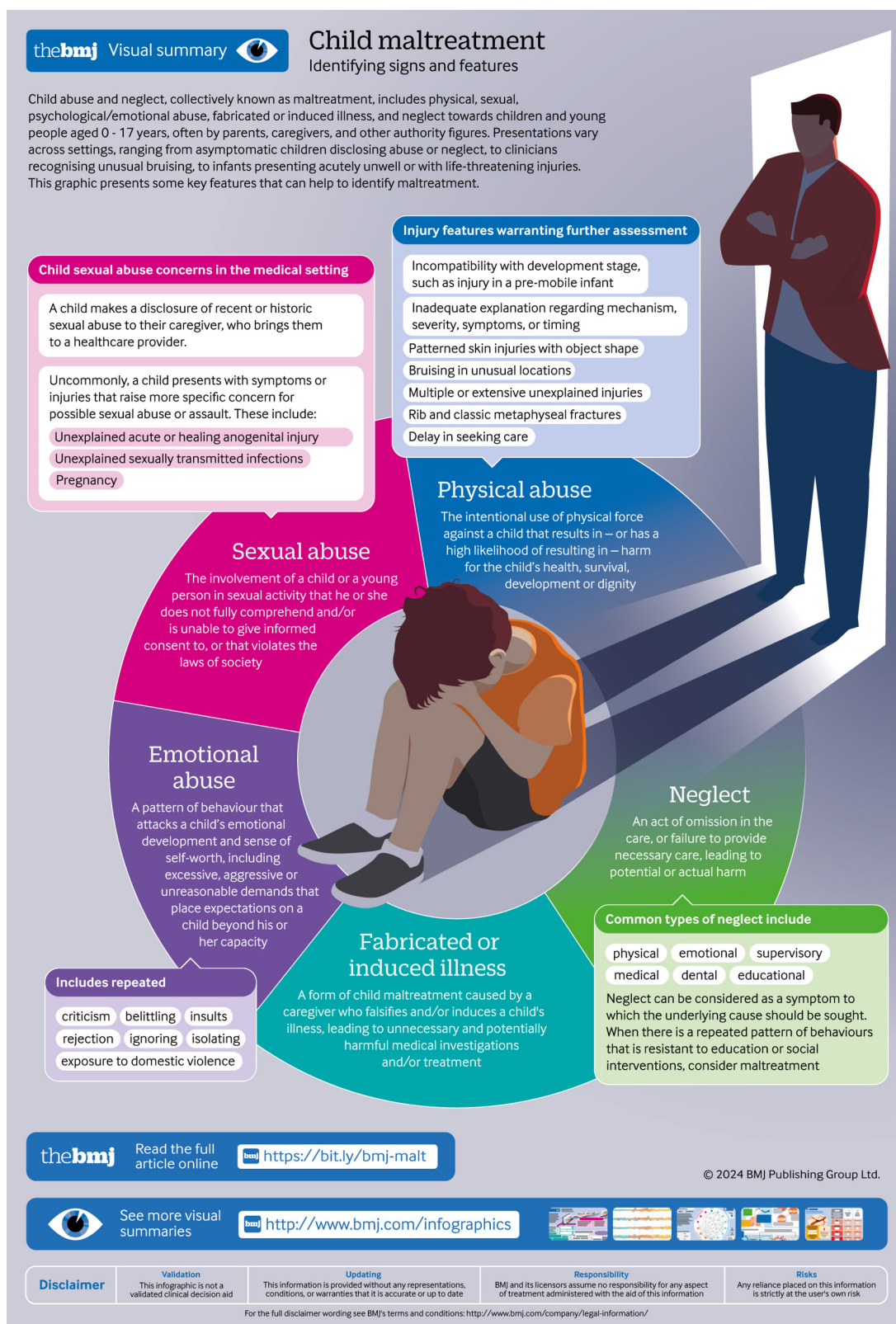
Children (Toronto, Canada), which cares for children and young people under the age of 18 years who have experienced, are experiencing, or may be at risk of experiencing commercial sexual exploitation or trafficking. They are currently a member of the Lotus Health Survivor Advisory Committee. The individual has also engaged and collaborated with other organisations/programs in the anti-human trafficking sector. Based on their feedback, more emphasis was placed on a youth-centered approach to medical care. This is evident throughout the article, but particularly in the sexual abuse section.

Education into practice

- Think about the last time you were concerned about possible child abuse. How would you manage it differently?
- What is your local child welfare or social services reporting process?
- What local specialists and mental health resources do you have available to support children and young people with suspected child abuse?

How this article was created

We performed Pubmed and OVID (Medline) searches, with keywords including “child abuse,” “child maltreatment,” “guidelines,” and “approach.” We reviewed English guidelines from North American and European medical associations and guidelines from international health organisations. We also appraised articles outlining approaches to child maltreatment. Additional articles on specific topics which were known to the authors provided supplemental information.



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